HCP Perception Survey: Opdivra in non-small cell lung cancer (EU, 2021)

Report date: 2021-11-04 | Prepared by: Daniel Hartwell

Executive Summary

This survey of 320 oncologists across Germany, France, Italy, Spain, United Kingdom examined how clinicians perceive Opdivra (nivoltrimab) for non-small cell lung cancer. Fieldwork took place in February 2021.

Overall HCP sentiment index for Opdivra in Germany: 0.69 (scale 0 to 1, assessed 2021). Respondents viewed the brand strongly positively.

Methodology

Respondents were oncologists who treat at least 6 patients per month with non-small cell lung cancer. The sample size was n=320. Interviews lasted 24 minutes and were conducted online.

Results are weighted to national prescribing volumes. Statistical significance was assessed at the 95% confidence level.

Sequencing after progression remains an open question; oncologists in United Kingdom reported using a different mechanism of action after Opdivra fails.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Regional variation within United Kingdom is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Treatment burden, including monitoring visits and missed maintenance doses, was described as a key factor in long-term adherence and persistence.

Dose modification guidance for Opdivra is generally followed, but oncologists said that practical monitoring of rash depends on local capacity and patient travel distance.

Medical education priorities include recognising rash early, communicating realistic expectations, and coordinating with primary care.

Respondents expect real-world persistence on Opdivra to be lower than in trials, mainly because of missed maintenance doses and the complexity of managing immune-related adverse events.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Opdivra and competing agents.

Subgroup analyses suggest that benefit with Opdivra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Key Findings

Adherence concerns centred on missed maintenance doses, particularly in patients with comorbidities and polypharmacy.

54% of respondents considered the efficacy of Opdivra clinically meaningful versus the current standard of care, with the strongest support in second-line use.

Respondents highlighted the mechanism of action of Opdivra (blocks the pd-1 receptor to restore t-cell activity) as a differentiator in non-small cell lung cancer, although cost and formulary access remain barriers.

Oncologists in Germany rated Opdivra tolerability profile as strongly positively, citing pneumonitis and rash as the main concerns among patients with non-small cell lung cancer.

Real-world evidence and patient-reported outcomes were requested by 40% of respondents before expanding use beyond first-line therapy.

Sequencing after progression remains an open question; oncologists in United Kingdom reported using a different mechanism of action after Opdivra fails.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Oncologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between PD-1 inhibitor options.

Safety monitoring focused on rash and infusion reactions, with dose interruption or discontinuation used for severe adverse events.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Opdivra has no direct substitute.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Medical education priorities include recognising rash early, communicating realistic expectations, and coordinating with primary care.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Opdivra and competing agents.

Subgroup analyses suggest that benefit with Opdivra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Tolerability and Safety Perceptions

Tolerability was assessed against pneumonitis and rash. Oncologists reported managing these events with dose adjustment and supportive care.

Safety perception differed by experience: respondents with more than 6 treated patients rated Opdivra more favourably.

Guideline committees are expected to review the evidence base for non-small cell lung cancer again next year, which could change eligibility criteria and referral patterns.

Dose modification guidance for Opdivra is generally followed, but oncologists said that practical monitoring of fatigue depends on local capacity and patient travel distance.

Respondents expect real-world persistence on Opdivra to be lower than in trials, mainly because of discontinuation after early adverse events and the complexity of managing rash.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Opdivra and competing agents.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Regional variation within United Kingdom is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Biosimilar and generic competition in adjacent classes is shaping price expectations, even where Opdivra has no direct substitute.

Sequencing after progression remains an open question; oncologists in United Kingdom reported using a different mechanism of action after Opdivra fails.

The unmet need in non-small cell lung cancer persists in patients who relapse or become refractory, where options are limited and outcomes remain poor.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Cost-effectiveness submissions typically model first-line use, and payers question the durability of benefit beyond the trial follow-up period.

Conclusions

Opdivra is perceived strongly positively in Germany. Priorities for the brand team are missed maintenance doses and evidence generation for non-small cell lung cancer.

Dose modification guidance for Opdivra is generally followed, but oncologists said that practical monitoring of rash depends on local capacity and patient travel distance.

Regional variation within Spain is notable: large centres report faster uptake and broader use than smaller regional hospitals.

Peer-to-peer education and congress data were cited as the most trusted sources when oncologists evaluate new evidence on PD-1 inhibitor agents.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Supply and logistics are not a concern for Opdivra, though cold-chain handling and nurse-led administration add workload in busy clinics.

Clinical guidelines in Spain recommend first-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Treatment burden, including monitoring visits and monitoring requirements, was described as a key factor in long-term adherence and persistence.

Registry data show that outcomes in non-small cell lung cancer depend heavily on early diagnosis, access to specialist care and adherence to maintenance therapy.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Subgroup analyses suggest that benefit with Opdivra is most pronounced in patients with a favourable biomarker profile and a lower comorbidity burden.

Safety monitoring focused on rash and fatigue, with dose interruption or discontinuation used for severe adverse events.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Segment	Respondents	Sentiment index
High-volume prescribers	128	0.73
Low-volume prescribers	192	0.66

Synthetic document generated for the Watson prototype. All brand names and figures are fictional.